10 — Real Data Plan: Replacing the Synthetic Data

> This is planning guidance, **not legal advice**. Confirm each step with the faculty advisor, the school's IRB, and the clinic's privacy officer. Contacts were checked against public websites and directories in November 2026. Confirm them before use.

1. What "real data" means for this project

The screening and referral logs this project measures **do not exist yet**, so no one can send them. They are created when a clinic uses the protocol. Public data can describe the community, but it cannot measure this clinic's screening or referral rates (P1–P4 in 07).

Data	Exists today?	How to get it	Replaces synthetic data?
Screening and referral logs	No. Created by running the protocol	Clinic partner + approvals (§2)	**Yes**
Baseline chart audit	Yes, in the clinic's records	Clinic partner + approvals (§2)	**Yes** (baseline weeks)
Public aggregate data (homeless count, health centers, food insecurity)	Yes	Download (§4)	No. Background, comparison, and simulation inputs only
Resource directory checks	Can be collected now	Phone calls (§7)	No. Supports 03 and 04

2. Path to real clinic data

Do not open patient records until steps 1–5 are done.

#	Step	Who	Result	See
1	Find a clinic partner and sponsor	Student	Letter or email of support	§5, Template A
2	Advisor approves the plan	Faculty advisor	Approval	00, 06
3	QI vs. research determination	School IRB	Determination letter or IRB approval	08 §1
4	Clinic onboarding	Clinic HR or compliance	Student or volunteer agreement, HIPAA training, confidentiality agreement	08 §2
5	Choose how data leaves the clinic	Clinic privacy officer	Written choice of route (§3)	08 §2
6	Adapt and verify materials	Student and sponsor	Resource directory verified; safety pathway signed off	03, 04
7	Baseline chart audit (weeks 1–4)	Student, on clinic systems	Baseline rows in `screening_log.csv`	07

#	Step	Who	Result	See
8	Go live with PDSA cycles	Clinic staff	Screening and referral logs, collected as visits happen	06
9	Analyze and report	Student or clinic staff	Aggregate results only	§3

Steps 1–5 can take weeks to months. Start partner calls and the IRB question early.

3. Handling the real data

Coded logs are not de-identified. They contain dates of service, so they are PHI under HIPAA even without names. Pick one route with the privacy officer (08 §2):

Route	Who handles row-level data	Paperwork	What leaves the clinic
1. Analyze on site	Student, as a clinic workforce member, on clinic-controlled storage	Student or volunteer agreement; HIPAA training	Aggregate results
2. Clinic runs the analysis	Clinic staff only	Privacy officer reviews outputs	Aggregate results
3. Limited Data Set	Student, off site	Data Use Agreement	Coded logs with dates

Run the analysis on the approved machine:

```
mkdir -p data/real                    # git-ignored
cp data/templates/*.csv data/real/    # blank logs with the right headers (or save clinic exports here)
# enter or export the data, then:
python analysis/analyze.py --data-dir data/real --out-dir data/real/output --strict
git status --short                    # data/real must not appear
```

- `--strict` stops on values that don't match the data dictionary. Fix the log and rerun.
- Keep `data/synthetic/` and `output/` as the demo. Never overwrite them with real data.
- Never commit anything from `data/real/`. Don't move it through cloud sync, email, or personal devices unless the clinic approves.
- `weekly_measures.csv` and the run charts contain unsuppressed weekly counts. The privacy officer reviews them before they leave the clinic.

4. Data that is already available

These sources are free and public, and need no approval. Use them for the background section, for comparisons, and to ground the simulation's assumptions. Do **not** present them as this project's results.

Source	What it gives	Where
Fairfax County Point-in-Time Count 2026 (OPEH)	1,365 people experiencing homelessness on Feb 4, 2026 (up 3% from 1,322 in 2025); demographics; shelter and housing inventory	fairfaxcounty.gov/homeless/point-time-count-2026

Source	What it gives	Where
HRSA Uniform Data System (UDS)	National, state, and health-center data, including patients experiencing homelessness. Starting with 2026 data, Table 6A adds counts of patients screened for upstream drivers of health and patients with food insecurity, housing instability, or financial insecurity. The first public release is likely in mid-2027	data.hrsa.gov
CDC PLACES (December 2025 release)	County and census-tract model estimates of 7 health-related social needs, including food insecurity, housing insecurity, and transportation barriers. Virginia is covered (it used the BRFSS social needs module in 2023). CDC cautions against using PLACES to evaluate programs	cdc.gov/places
Feeding America, Map the Meal Gap	County food insecurity estimates (data through 2024, released July 2026); dataset request form	map.feedingamerica.org
Capital Area Food Bank, Hunger Report 2026	NORC survey of 3,804 adults (May 2026): 27% of Fairfax County households food insecure; 38% across the region	capitalareafoodbank.org

General-population rates likely understate needs among people experiencing homelessness. If you use them in the simulation, say so and cite the source.

5. Who to contact

Possible clinic partners

Organization	Why	Phone	Email
New Hope Housing, Mobile Medical Outreach	Mobile care for people experiencing homelessness; closest match	703-799-2293	admin@newhopehousing.org
Fairfax County Health Department, Homeless Healthcare Program	Nurse practitioners in shelters, drop-in centers, and street outreach	703-246-2411 (main line; ask for the program)	
GMU MAP Clinics	Academic clinics, including shelter-based sites	703-993-5880	mapclinic@gmu.edu
Neighborhood Health (FQHC)	Health center; may share aggregate screening counts	703-535-5568	partners@nhnva.org
HealthWorks for Northern Virginia (FQHC)	Health center with a Sully site (13800 Wall Rd, Herndon)	703-443-2000	

Data holders

Organization	Ask for	Phone	Email / mail
Fairfax County Office to Prevent and End Homelessness (OPEH)	Aggregate Point-in-Time or HMIS figures beyond the dashboard (Template B)	703-246-5000	OPEHGeneralMail@fairfaxcounty.gov · 3700 Pender Drive, Fairfax, VA 22030
Fairfax County FOIA Office	Existing aggregate records only (Template C)	703-324-7052	FOIA@fairfaxcounty.gov · 12000 Government Center Pkwy, Suite 551, Fairfax, VA 22035
HRSA UDS Support Center	Help reading UDS data	866-UDS-HELP (866-837-4357)	udshelp330@bphcdata.net
Feeding America	Map the Meal Gap dataset		Online request form (feedingamerica.org, Map the Meal Gap)
Your school's IRB or QI office	QI determination (step 3)		

Questions to ask a clinic

1. Do you document on paper or in an EHR? Which one, and does it work offline?
2. Do you screen for social needs now? Which tool, who asks, and how often?
3. How many unique adult patients do you see per week, and how many visits?
4. Are you an FQHC or Health Care for the Homeless grantee that reports UDS?
5. How do you track referrals and follow-up?
6. Would you sponsor a student QI project? What onboarding is required?

6. Outreach templates

Replace everything in [brackets].

Template A — Clinic partnership (email or letter)

> **Subject:** Student quality improvement project on social needs screening
>
> Dear [Name],
>
> I am a [program] student at [school], supervised by [advisor name, title]. For my quality improvement project, I have prepared a ready-to-use package for standard social needs screening at a mobile clinic serving people experiencing homelessness. It includes a short questionnaire based on the CMS Accountable Health Communities tool, a screening protocol, staff training, a Fairfax County resource directory, and a measurement plan.
>
> I am looking for a clinic partner to pilot it for about [16] weeks. The project would:
>
> - Screen adult patients for housing, food, transportation, utilities, safety, health literacy, and ID needs, and offer referrals.
> - Measure the share of eligible visits screened and referred, using coded logs that stay under the clinic's control.

> • Start only after my school's QI determination and your clinic's approval.
>
> Could we set up a 15-minute call? I would also like to learn how your clinic documents social needs today.
>
> Thank you, [Name] · [phone] · [email]

Template B — Aggregate data request

> **Subject:** Request for aggregate data for a student QI project
>
> Dear [Name],
>
> I am a [program] student at [school] working on a quality improvement project about social needs screening for adults experiencing homelessness in Fairfax County. I am asking for **aggregate figures only**, with no patient-level data, for [time period], if they already exist:
>
> 1. [Number of adult patients or visits]
> 2. [Number screened for social needs, and the tool used]
> 3. [Number who screened positive for food, housing, or transportation needs]
> 4. [Number referred]
>
> Counts under [5] may be suppressed. I will use the figures only as background for my [course project/ thesis] and will cite them as you direct.
>
> Thank you, [Name] · [phone] · [email] · Advisor: [name, email]

Template C — Virginia FOIA request (existing records)

> **To:** Fairfax County FOIA Office, FOIA@fairfaxcounty.gov
>
> Under the Virginia Freedom of Information Act (Va. Code § 2.2-3700 et seq.), I request copies of the following existing public records: [for example, the most recent annual report or performance summary for the Health Department's Homeless Healthcare Program showing aggregate numbers of people served]. I am a Virginia resident. I prefer electronic copies. Please tell me in advance if costs will exceed $[amount].
>
> [Name] · [mailing address] · [email]

Virginia FOIA covers existing records only. Agencies do not have to create new reports, and patient health records are exempt. Requesters must be Virginia citizens or qualifying media.

7. If no partner is available in time

- Keep the synthetic data and label it **simulated** everywhere.
- Replace the example probabilities in `analysis/generate_synthetic_data.py` with sourced estimates, and cite each one. Prefer published studies of patients experiencing homelessness, and use the §4 data for local context.

- Ask the advisor whether an implementation-ready package with simulated results meets the course requirements.
- Collect real data that involves no patients now. Call every resource in 04, confirm hours, eligibility, and intake steps, and record the date in the “Verified” column.

8. Checklist

- [ ] Clinic partner and sponsor confirmed
- [ ] Advisor approval
- [ ] QI/IRB determination letter on file
- [ ] Clinic onboarding complete
- [ ] Route for getting data out of the clinic chosen in writing
- [ ] Resource directory verified by phone
- [ ] Baseline audit complete (weeks 1–4)
- [ ] Analysis run in `data/real/` on approved storage; outputs reviewed by the privacy officer